45 W Crossville Rd, Roswell GA 30075 - 6931
Tel: 770 800-4357 x , Fax: 470 709-5810 x

PROGRESS NOTE

Patient First Name:	Patient Last Name:	Date of Birth:	Sex:
Aidan	Wilson	02-20-2007	Male
Attending Provider:	**Referring Provider:**	**Visit Date:**	**Chart No.:**
Edyta Chmiel, NP		01-08-2025	SCL09899
Appointment Location:	**Appointment Location Address:**		
NS - Roswell	45 W Crossville Rd, Roswell GA 30075 - 6931		

Reason For Visit: Follow up on low back pain and mid back pain.

Chief Complaint: Follow Up
History of Present Illness

Follow Up Patient presents to clinic via telemedicine accompanied by his father. Patient reports improvement in neck 1/10, mid back 2/10 and low back pain 2/10 in intensity. Patient reports.

Past Medical History
Asthma ()

Surgical History
Achilles Tendon

Current Medication
No Known Current Medication

Allergy
No Known Drug Allergies.

Social History
Family: He is single.

Review of Systems
Constitutional Symptoms: Denies fever, chills, hot flashes, night sweats and weight loss.

Vitals

TELEMEDICINE APPOINTMENT

Physical Examination

TELEMEDICINE APPOINTMENT

Assessment
ICD: Follow up (Z09)
Lumbago M54.50
Lumbar radiculopathy M54.16
Thoracic pain
Cervicalgia

Patient presents accompanied by his father via telemedicine for follow up on mid back, neck and low back pain. Patient reports improvement in neck, mid back and low back pain. He is attending physical therapy.

Imaging:

MRI of thoracic spine: Unremarkable
MRI of Lumbar spine: At L4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac.

This patient has suffered a significant motor vehicle accident. Symptoms today are compatible with significant cervical whiplash.

I spent 25 minutes today preparing the patient's chart, taking history, discussing diagnosis and recommend treatment options for management of symptoms, detailing recommended intervention and documenting in the EHR.

Plan
- Continue physical therapy for mid back and low back pain.

- Follow up as needed.

- Patient notes improvement in neck, mid and low back pain. Thus, I will discharge from active care at this time. The patient counselled that symptoms may continue to occur intermittently, likely for at least 3-6 months after the initial injury. The patient advised to contact the clinic if pain or other symptoms worsen for consideration of additional treatment options as warranted.

DIAGNOSTIC TESTING

CPT Codes:
Office O/p Est Mod (99214)

Follow up: As Needed

Edyta Chmiel, NP
This has been electronically signed by Edyta Chmiel, NP on 01-08-2025.

FirstVisit MEDICAL SERVICES

FIRST VISIT MEDICAL SERVICES PA
390 NE 191st St STE 8349
Miami, Florida - 33179-3899
Tel: 754-315-7082, Fax: 754-315-7082

Patient	: **Aidan Wilson**	Record Id :	**PAT8081**
Date of Birth	: **Feb 20, 2007**	Age / Gender :	**17** / **M**
Provider	**: Lindsay Lawhorne, AGNP-C**	Date of Visit	**: Nov 07, 2024**
Visit Type	**: New Patient Visit**		

Chief Complaints

neck pain, lower back pain, and headaches

Related To

Auto Accident - Date: Oct 25, 2024; Place: Georgia; Qualifier: (454) Initial Treatment

History of Present Illness

This is a 17 year old male patient, who presents with his father, complaining of neck pain, lower back pain, and headaches following a motor vehicle accident, which occurred on 10/25/24. The patient was a restrained front passenger and the impact occurred on the rear end of the vehicle. The airbags did not deploy. He was able to exit the vehicle independently and there was no loss of consciousness.

Neck pain is rated a 7 on the 0-10 pain scale and is intermittent. It is described as aching. The pain does not radiate. Aggravating factors include movement. Relieving factors include rest. Denies any numbness or tingling in extremities.

Lumbar back pain is rated a 5 on the 0-10 pain scale and is intermittent. It is described as aching. The pain does not radiate. Aggravating factors include movement. Relieving factors include rest. The patient denies any loss of bowel or bladder function.

Patient complains of headaches which began shortly after the accident occurred. Pain is rated a 6 on the 0-10 pain scale and is intermittent. It is described as aching. Aggravating factors include none. Relieving factors include none. Denies any dizziness, vision changes, nausea/vomiting, or photosensitivity.

The patient denies having any of the above symptoms prior to the accident. He was evaluated in the Emergency Department following the accident and is currently receiving chiropractic treatment.

Past Medical History

No pertinent past medical history noted.

Family and Social History

Family History

Non-contributory.

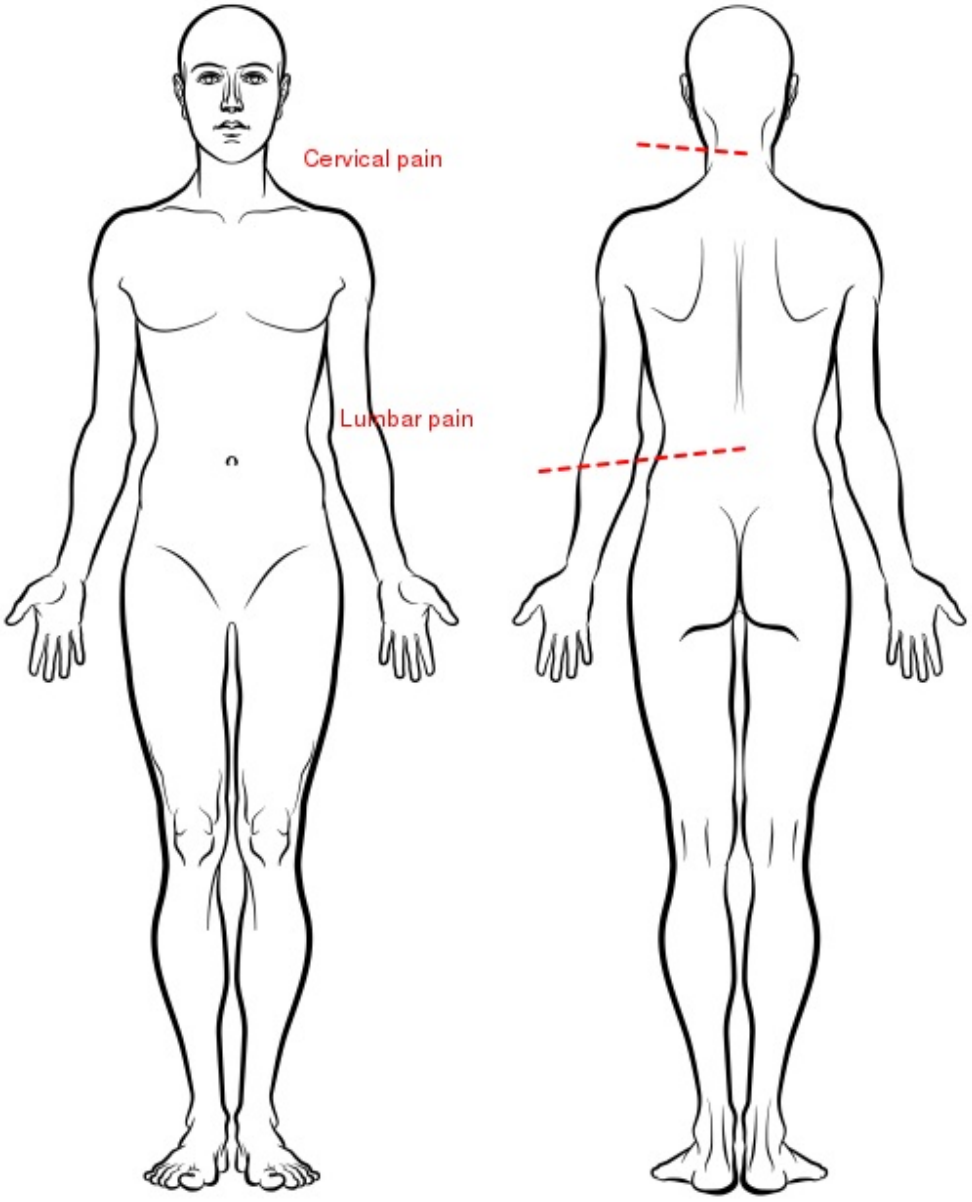
Social History

Patient is not employed.

Patient	: **Aidan Wilson**	Record Id :	**PAT8081**
Date of Birth	: **Feb 20, 2007**	Age / Gender :	**17** / **M**
Provider	**: Lindsay Lawhorne, AGNP-C**	Date of Visit	**: Nov 07, 2024**
Visit Type	**: New Patient Visit**		

Review of Systems

12 -point ROS conducted and all are negative unless otherwise specified in HPI.



Physical Examination

Vital Signs

Patient	: **Aidan Wilson**	Record Id :	**PAT8081**
Date of Birth	: **Feb 20, 2007**	Age / Gender :	**17** / **M**
Provider	**: Lindsay Lawhorne, AGNP-C**	Date of Visit	**: Nov 07, 2024**
Visit Type	**: New Patient Visit**		

Deferred due to telemedicine visit.

Physical exam completed with limitations due to nature of telemedicine visit.

General

The patient appears to be in moderate distress.

Cervical Spine

Pain noted with cervical spine flexion? Yes
Pain noted with extension of cervical spine? No
Pain noted with left lateral rotation of cervical spine? No
Pain noted with right lateral rotation of cervical spine? No
Pain noted with right lateral flexion of cervical spine? No
Pain noted with left lateral flexion of cervical spine? No

Lumbar Spine

Pain noted with lumbar flexion? No
Pain noted with lumbar extension? No
Pain noted with left lateral flexion? No
Pain noted with right lateral flexion? No

Neurological

Patient is alert and oriented x 4.

Assessment Notes

The Stanford Concussion Questionnaire was used to assess risk factors consistent with a traumatic brain injury.

Calculated score: 2

Note: Scores above zero are considered abnormal.

A trauma questionnaire was administered and negative for signs/symptoms of PTSD.

Diagnoses

FirstVisit MEDICAL SERVICES

FIRST VISIT MEDICAL SERVICES PA
390 NE 191st St STE 8349
Miami, Florida - 33179-3899
Tel: 754-315-7082, Fax: 754-315-7082

Patient : **Aidan Wilson**
Date of Birth : **Feb 20, 2007**
Provider **: Lindsay Lawhorne, AGNP-C**
Visit Type **: New Patient Visit**

Record Id : **PAT8081**
Age / Gender : **17** / **M**
Date of Visit **: Nov 07, 2024**

Diagnosis	Code	Comments
Headache, unspecified	R51.9	
Low back pain, unspecified	M54.50	
Cervicalgia	M54.2	

Rx

Prescription	Directions	Details
ibuprofen 800 mg tablet [IBUPROFEN]	1 tablet(s) every 8 hours as needed for 30 Day(s), oral	Dispense :90 Tablet, Refill :0,Generic Allowed

Instructions

Please visit your nearest Emergency Department for any new or worsening symptoms, including, but not limited to: headaches that worsen, seizures, focal neurological signs, increased drowsiness, difficulty awakening, repeated vomiting, slurred speech, difficulty recognizing people or places, increased confusion or irritability, increased weakness or numbness in arms and/or legs, unusual change in behavior or change in level of consciousness, new or worsening shortness of breath, new or worsening chest pain, or any general feeling of being unwell.

Recommend TBI evaluation.

Recommend referral to pain management for evaluation and treatment of cervical and lumbar spine pain.

Prescription sent to pharmacy on file.

Recommend conservative therapy, exercise, and stretching activities throughout recovery as tolerated.

Attestation

These services were provided under physician supervision.

Referrals

FIRST VISIT MEDICAL SERVICES PA
390 NE 191st St STE 8349
Miami, Florida - 33179-3899
Tel: 754-315-7082, Fax: 754-315-7082

Patient : **Aidan Wilson**
Date of Birth : **Feb 20, 2007**
Provider **: Lindsay Lawhorne, AGNP-C**
Visit Type **: New Patient Visit**

Record Id : **PAT8081**
Age / Gender : **17** / **M**
Date of Visit **: Nov 07, 2024**

Referral To	Reason
Specialty Referral	MVA

Lindsay Lawhorne, AGNP-C

Lindsay Lawhorne, AGNP-C

Electronically Signed By Lindsay Lawhorne, AGNP-C on Nov 07, 2024 06:23 PM US/Eastern

Electronically Co-Signed By Katherine Wiegman, MD on Nov 08, 2024 07:47 AM

Co-Signer Note: Katherine Wiegman

ADVANCED IMAGING CENTERS

PATIENT:	WILSON, AIDAN
DOB:	2/20/2007
FILE #:	RAM896786
PHYSICIAN:	HEINIS, RICARDO
EXAMS:	MRI T-SPINE W/O CONTRAST
DATE:	11/25/2024 1:00:00 PM

MRI OF THE THORACIC SPINE:

HISTORY: Mid back pain following a motor vehicle collision dated 10/25/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: Evaluation of the thoracic spine demonstrates normal kyphotic curvature. No evidence for abnormal mass or fluid collection is seen. There is no evidence for fracture. There is no evidence for prevertebral or paravertebral mass or fluid collection. Examination of the thoracic cord demonstrates that there is normal signal. The adjacent soft tissues demonstrate no significant abnormalities.

Segmental analysis of the thoracic spine is as follows:

At T1-2, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T2-3, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T3-4, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T4-5, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T5-6, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T6-7, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T7-8, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T8-9, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T9-10, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T10-11, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

At T11-12, there is no evidence for disc herniation, canal stenosis, or foraminal stenosis.

IMPRESSION:

1. Unremarkable MRI of the thoracic spine.

The definitions in this report, including definitions of disc bulge, herniation, protrusion, and extrusion, are from the following peer reviewed journal: Lumbar Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525-2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

SEAN M. MAHAN, M.D.
ELITERAD RADIOLOGY GROUP
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SMM/DW

ADVANCED IMAGING CENTERS

PATIENT:	WILSON, AIDAN
DOB:	2/20/2007
FILE #:	RAM896787
PHYSICIAN:	HEINIS, RICARDO
EXAMS:	MRI L-SPINE W/O CONTRAST
DATE:	11/25/2024 1:30:00 PM

MRI OF THE LUMBAR SPINE:

HISTORY: Low back pain following a motor vehicle collision dated 10/25/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: The conus medullaris appears normal. The lordotic curvature of the lumbar spine is preserved. No evidence for abnormal solid or cystic lesions is identified. No prevertebral or paravertebral masses or fluid collections are seen and there is no evidence for abnormal marrow replacing lesion.

Segmental analysis of the lumbar spine is as follows:

At L1-2, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L2-3, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L3-4, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At L5-S1, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

IMPRESSION:

1. At L4-5, there is bulging of the disc. This results in an anterior impression on the thecal sac.

The definitions in this report, including definitions of disc bulge, disc herniation, protrusion, and extrusion, are from the following peer reviewed journal: Lumbar Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525 2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

SEAN M. MAHAN, M.D.
ELITERAD RADIOLOGY GROUP
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SMM/JN

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

Initial Exam

Patient Name: Aidan Wilson
Date of Birth: 2/20/1977

Provider: Rita Patel, DC
Date of Service: 10/29/2024

SUBJECTIVE

HISTORY OF PRESENT ILLNESS

He has never experienced this condition before.

PRIMARY COMPLAINT

Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT

Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT

Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT

Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

PAST, FAMILY, SOCIAL HISTORY

Surgeries/Hospitalizations: None reported
Allergies: None reported
Medications: None reported
Prior conditions: None
Works at Texsource. Single no children. Likes to pl;ay video games and work out.

REVIEW OF SYSTEMS

ALLERGIC-IMMUNOLOGIC: Allergies
CARDIOVASCULAR: Dizziness
EYES: Glasses/Contacts and Blurred vision

GASTROINTESTINAL: Constipation and Heartburn
MUSCULOSKELETAL: Stiffness, Muscle pain, Neck pain, Stiff neck and Back pain
NEUROLOGICAL: Headaches and Light Headed/Dizzy
PSYCHIATRIC: Anxiety and Nervousness
RESPIRATORY: Asthma

Collision occurred on 10/25/2024 at approximately 12:22 PM.
Symptoms reported: None.

Mr. Wilson was a passenger in the vehicle, a 2019 Acura Rdx, at the time of the collision. There was rear damage to the vehicle. The collision happened while he was stopped at Club drive in Lawrenceville , GA and occurred during the day. The weather was clear. Street surface was dry. The impact on the vehicle was at the rear end. Upon impact, the brakes were locked, and the car moved 1 to 5 feet. Mr. Wilson was seated in the vehicle as a front passenger and was wearing a seat belt with the shoulder harness and with the headrest in up position. The vehicle is equipped with air bags and they did not deploy. Mr. Wilson didn't see the impact coming. Upon impact, his head was looking down and he was thrown forward. Mr. Wilson remembers the collision happening.
* Medications given: Cyclobenzaprine.

The patient reports to this office due to injuries received in an MVC

OBJECTIVE

VITALS

Height: 65 in Weight: 120 lbs BMI: 20.0
Temperature: 98.4°F Pulse rate: 78 BPM (relaxed) SpO_2: 98%.
Blood Pressure (seated): right -N/A left-120/86 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI.

Tenderness

Left Side
- Cervical Spine (moderate)
- Shoulder (moderate)
- Thoracic Spine (moderate)
- Lower Back (moderate)

Right Side
- Cervical Spine (moderate)
- Shoulder (moderate)
- Thoracic Spine (moderate)
- Lower Back (moderate)

Spasm/Hypertonicity

Left Side
- Cervical Spine (moderate)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate)

Right Side
- Cervical Spine (moderate)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate)

GAIT

Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Foraminal compression test[1]	Positive	WNL	

Maximal foraminal compression test[2]	WNL	Positive	
Shoulder depression test[3]	Positive	Positive	
Kemp's test[4]	Positive	Positive	
Lasegue's test[5]	Positive	WNL	
Milgram's test[6]	Positive	WNL	
Spinal percussion test-thoracic[7]	Positive	Positive	
Spinal percussion test-lumbar[8]	Positive	Positive	

[1]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[2]Increased peripheral pain could indicate radiculopathy or IVF encroachment. Local neck pain could indicate joint or muscle damage.

[3]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[4]Perhipheral pain in to the lower extremities indicates nerve root compression. Local pain may indicate sprain or strain.

[5]Limitation due to pain is suggestive of nerve root or sciatica irritation.

[6]Lower back pain may indicate a herniated disc.

[7]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[8]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

REFLEXES

Biceps, Tricep, Brachioradialis and Patellar are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Right rotation	75° (93% of normal)	Pain, Spasm, Guarding	

Flexion, Extension, Left lateral flexion, Right lateral flexion and Left rotation are within normal limits.

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	55° (91% of normal)	Spasm, Guarding	
Extension	20° (80% of normal)	Pain, Spasm, Guarding	
Left rotation	25° (83% of normal)	Pain, Spasm, Guarding	

Left lateral flexion, Right ateral flexion and Right rotation are within normal limits.

MUSCLE STRENGTH

Upper Muscle Strength

Location	Left	Right	Comment
Cervical flexors/extensors	4/5	4/5	
Cervical lateral flexon left/right	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

Lower Muscle Strength

Location	Left	Right	Comment
Hip flexors	4/5	4/5	
Hip extensors	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.
There are no contraindications to chiropractic care.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

LONG-TERM GOALS

- Decrease pain to pre-injury status.
- Restore tolerance to normal activities of daily living.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

There is no past medical history that would contribute to the current exam findings. The current complaints are a result of this MVC.

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 2 stages of care; Acute and Rehabilitative.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 99213 Re-evaluations will be performed once every 9 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 99213 Re-evaluations will be performed once every 9 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

- New Patient evaluation and management service, 99203, was provided today.

The patient presents today for a New Patient Exam. The course of care and treatment plan was discussed with the patient. The patient had an opportunity to ask questions and a treatment plan was given to the patient which will be 3 times per week for 3 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 10/29/2024 01:14 PM

Addenda

The patient was referred to this office by another healthcare provider.

Electronic Signature: Patel, Rita DC

Added on Oct 29, 2024

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/1977

Provider: Ricardo Heinis, DC
Date of Service: 11/2/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient had focal discomfort in his **neck and upper back region**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI**.**

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
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5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 2 stages of care; Acute and Rehabilitative.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 99213 Re-evaluations will be performed once every 9 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 99213 Re-evaluations will be performed once every 9 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- **CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique, to reduce pain reported at initial encounter by 25%.**
- **Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.**
- **Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.**
- **Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.**
- **Diagnostic Tests: X-ray thoracic 2 views (72070) was provided.**
- **Diagnostic Tests: X-ray lumbosacral 2-3 views (72100) was provided.**
- **Diagnostic Tests: X-ray cervical 4-5 views (72050) was provided.**

Patient **completed X-rays today**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/04/2024 04:14 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/1977

Provider: Rita Patel, DC
Date of Service: 11/5/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI.

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)

- Lower Back (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique **was provided specifically to C3**; **C5**; **T3**; **T4**; **T6**; **T11**; **T12**; **L1**; **L2**; **L4**; **L5**; **SI**, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99212, was provided today.**

Review x-rays with the patient. **The patient had an opportunity to ask questions and understand the findings**. **Due to positive findings on those** X-rays, **the patient will be referred for an MRI of the Cervical and Lumbar spine for further assessment**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 11/05/2024 11:07 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/1977

Provider: Amanda Ruiz, DC
Date of Service: 11/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI.

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of **Instrument adjusting**, diversified technique was provided specifically to **C1**; **C2**; **C7**; T3; T4; **T5**; T12; L5; **Pelvis**; **Ilium**, to **reduce pain reported at initial encounter by 50%**.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)

Provider Name: Amanda Ruiz, DC
Date: 11/09/2024 10:32 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/1977

Provider: Ricardo Heinis, DC
Date of Service: 11/15/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient has soreness and tightness in the upper and lower back.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI**.**

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C2; **T2**; T3; T4; T5; **L4**; L5; Pelvis; Ilium, to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/15/2024 02:27 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/1977

Provider: Ricardo Heinis, DC
Date of Service: 11/16/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient **stated that he only felt noticeable discomfort** in **his** upper back **this morning**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI**.**

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C2; T2; T3; T4; T5; **T6**; L4; L5, to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/16/2024 12:39 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Rita Patel, DC
Date of Service: 11/19/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI.

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)

- Lower Back (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C2; **C5**; **T2**; T6; **L2**; **L4**; **SI**, to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 11/19/2024 10:46 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Amanda Ruiz, DC
Date of Service: 11/23/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI.

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C2; **C3**; **T3**; **T4**; **T5**; **L5**; **Pelvis**, to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 11/23/2024 10:12 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Ricardo Heinis, DC
Date of Service: 11/26/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient has not had any changes from his last visit.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI**.**

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C3**; **C4**; T4; T5; **T6**; L5; Pelvis, to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/26/2024 11:59 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Ricardo Heinis, DC
Date of Service: 11/30/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient **states that he is having more range of motion**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI**.**

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C3; C4; T4; T5; T6; T7; T8, to reduce pain reported at initial encounter by 50%.**
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/30/2024 11:55 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Amanda Ruiz, DC
Date of Service: 12/7/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis; Sacrum; Ilium; SI.

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

- Thoracic Spine (Moderate)
- Lower Back (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, **activator technique** was provided specifically to C1; C3; C4; T4; T5; T6, to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 12/07/2024 11:00 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Rita Patel, DC
Date of Service: 12/10/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: **From 5 to 2/10 (30% decrease)**
  Described as **occasional (up to 25% of day)** of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: **From 4 to 2/10 (20% decrease)**
  Described as **intermittent (up to 50% of day)** of **slight to moderate intensity**; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: **From 7 to 5/10 (20% decrease)**
  Described as **frequent (up to 75% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: **From 8 to 5/10 (30% decrease)**
  Described as **intermittent to frequent (up to between 50% and 75% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Feels more pain when he is lying down and getting up.

OBJECTIVE

VITALS
Height: 65 in Weight: 120 lbs BMI: 20.0
Temperature: 98.3°F Pulse rate: 60 BPM (relaxed) SpO_2: 98%.
Blood Pressure (seated): right -N/A left-116/70 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C5; T2; T6; L1; L2; L5; Pelvis**. No findings: OCC; C1; C2; C4; C6; C7; T1;**

T3; T4; T5; T7; T8; T9; T10; T11; T12; L3; L4; Sacrum; Ilium; SI.

Tenderness

Left Side

- Cervical Spine **(decreased 10-20% to mild)**
- Shoulder **(decreased 20-35% to minimal)**
- Thoracic Spine **(decreased 10-20% to mild)**
- Lower Back **(decreased 5-10% to mild-moderate)**

Right Side

- Cervical Spine **(decreased 10-20% to mild)**
- Shoulder **(decreased 20-35% to minimal)**
- Thoracic Spine **(decreased 10-20% to mild)**
- Lower Back **(decreased 5-10% to mild-moderate)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 20-35% to minimal)**
- Thoracic Spine **(decreased 35-50% to minimal)**
- Lower Back **(decreased 5-10% to mild-moderate)**

Right Side

- Cervical Spine **(decreased 20-35% to minimal)**
- Thoracic Spine **(decreased 35-50% to minimal)**
- Lower Back **(decreased 5-10% to mild-moderate)**

GAIT

Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Foraminal compression test, Maximal foraminal compression test, Shoulder depression test, Kemp's test, Lasegue's test, Milgram's test, Spinal percussion test-thoracic and Spinal percussion test-lumbar are within normal limits.

REFLEXES

Biceps, Tricep, Brachioradialis and Patellar are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Flexion, Extension, Left lateral flexion, Right lateral flexion, Left rotation and **Right rotation** are within normal limits.

Lumbar ROM - estimated

Flexion, **Extension**, Left lateral flexion, Right ateral flexion, **Left rotation** and Right rotation are within normal limits.

MUSCLE STRENGTH

Upper Muscle Strength

Location	Left	Right	Comment
Cervical flexors/extensors	4/5	4/5	
Cervical lateral flexon left/right	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

Lower Muscle Strength

Location	Left	Right	Comment
Hip flexors	4/5	4/5	
Hip extensors	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 0% to 80% of goal achieved.**
- Restore tolerance to normal activities of daily living: **Progressed from 0% to 80% of goal achieved.**

Patient is **showing improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; C5; T2; T6; L1; L2; L5; Pelvis, to reduce pain reported at initial encounter by 100%.**
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Reexam today. The new care plan will be 2 times per week for 4 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 12/10/2024 10:54 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Ricardo Heinis, DC
Date of Service: 12/14/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 2/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 2/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient was feeling some tightness in the cervical and **upper thoracic regions**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C3; C5; T2; T6; L1; L2; L5; Pelvis**.**

Tenderness

Left Side

- Cervical Spine (Mild)
- Shoulder (Minimal)

Right Side

- Cervical Spine (Mild)
- Shoulder (Minimal)

- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; **C4**; **T3**; T6; **L2**; **L4**; L5; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to **reduce pain reported at initial encounter by 100%**.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to **Back**, Neck, to **reduce pain reported at initial encounter by 100%**.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to **reduce pain reported at initial encounter by 100%**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/14/2024 11:41 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Amanda Ruiz, DC
Date of Service: 12/21/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 2/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 2/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C5; T2; T6; L1; L2; L5; Pelvis.
Tenderness

Left Side

- Cervical Spine (Mild)
- Shoulder (Minimal)

Right Side

- Cervical Spine (Mild)
- Shoulder (Minimal)

- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, **Thompson technique** was provided specifically to C3; C4; T3; T6; L2; L4; L5; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 12/21/2024 10:59 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Ricardo Heinis, DC
Date of Service: 12/28/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 2/10
  Described as occasional (up to 25% of day) of slight to moderate intensity; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 2/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient is staying stable with his upper to mid back

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C5; T2; T6; L1; L2; L5; Pelvis**.**

Tenderness

Left Side

- Cervical Spine (Mild)
- Shoulder (Minimal)

Right Side

- Cervical Spine (Mild)
- Shoulder (Minimal)

- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; C4; T3; **T4**; **T6**; L4; L5, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/29/2024 08:55 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Rita Patel, DC
Date of Service: 1/2/2025

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: **From 2 to 1/10 (10% decrease)**
  Described as **infrequent** of **minimal intensity (requires very little or no modification to ADL)**; increased by standing; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 2/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by standing; decreased by rest.

Patient **states his mid to low back bothers him especially when changing from sitting or laying down** to **standing**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C5; T2; T6; L1; L2; L5; Pelvis; **T11; T12.**
Tenderness

Left Side

- Cervical Spine (Mild)

Right Side

- Cervical Spine (Mild)

- Shoulder (Minimal)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

- Shoulder (Minimal)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
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11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; C4; T3; T4; T6; **T12**; **L1**; L4; L5, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 01/02/2025 10:50 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Rita Patel, DC
Date of Service: 1/4/2025

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain
  Described as of ; increased by standing; decreased by **Chiropractic care** and rest.

SECOND COMPLAINT
Neck

- Aching pain: 2/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity; increased by ADL's; decreased by **Chiropractic care** and rest.

THIRD COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain: **From 5 to 2/10 (30% decrease)**
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of **slight intensity (requires some modification to ADL)**; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Low back, Mid back

- Stabbing, Sharp pain: 5/10
  Described as **intermittent (up to 50% of day)** of **slight to moderate intensity**; increased by standing; decreased by **Chiropractic care** and rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C5; T2; T6; T12; L1; Pelvis**. No findings: T11; L2; L5**.
Tenderness

Left Side

- Cervical Spine (Mild)
- Shoulder (Minimal)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Shoulder (Minimal)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Minimal)
- Thoracic Spine (Minimal)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
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11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; C4; T3; T4; T6; T12; L1; L4; L5, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 01/04/2025 10:40 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Aidan Wilson
Date of Birth: 2/20/2007

Provider: Ricardo Heinis, DC
Date of Service: 1/14/2025

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 2/10
  Described as of ; increased by ADL's; decreased by Chiropractic care and rest.

SECOND COMPLAINT
Right scapula, Left scapula, Upper back

- Sharp pain
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight intensity (requires some modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back, Mid back

- **Aching** pain: **From 5 to 2/10 (30% decrease)**
  Described as **infrequent** of **minimal intensity (requires very little or no modification to ADL)**; increased by standing; decreased by Chiropractic care and rest.

FOURTH COMPLAINT
Head

- Aching pain
  Described as increased by standing; decreased by Chiropractic care and rest.

Patient expressed tat he had been doing fine and with good range of motion.

OBJECTIVE

VITALS
Height: 65 in Weight: 120 lbs BMI: 20.0
Temperature: 98.3°F Pulse rate: 52 BPM (relaxed) SpO_2: 99%.
Blood Pressure (seated): right -N/A left-101/59 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS
Spinal asymmetry/fixation: C3; C5; T2; T6; T12; L1; Pelvis**.**

Tenderness

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 10-20%)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 10-20%)**

- Lower Back **(decreased 10-20% to minimal)**

- Lower Back **(decreased 10-20% to minimal)**

Spasm/Hypertonicity

Left Side

- Thoracic Spine **(decreased 5-10%)**
- Lower Back **(decreased 10-20% to minimal)**
- Cervical Spine (Minimal)

Right Side

- Thoracic Spine **(decreased 5-10%)**
- Lower Back **(decreased 10-20% to minimal)**
- Cervical Spine (Minimal)

GAIT

Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Foraminal compression test, Maximal foraminal compression test, Shoulder depression test, Kemp's test, Lasegue's test, Milgram's test, Spinal percussion test-thoracic and Spinal percussion test-lumbar are within normal limits.

REFLEXES

Biceps, Tricep, Brachioradialis and Patellar are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Flexion, Extension, Left lateral flexion, Right lateral flexion, Left rotation and Right rotation are within normal limits.

Lumbar ROM - estimated

Flexion, Extension, Left lateral flexion, Right ateral flexion, Left rotation and Right rotation are within normal limits.

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and **Cervical lateral flexon left/right** are within normal limits bilaterally.

Lower Muscle Strength

Hip flexors and **Hip extensors** are within normal limits bilaterally.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

Patient continued to hold his adjustments during these final visits.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
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7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Prognosis has changed from **Fair to Good** - Symptomatic and functional recovery is expected in approximately 8-12 weeks.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 80% to 90% of goal achieved.**

Patient is **showing improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C2**; C3; L5; **Pelvis**, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Final exam today. The patient has reached maximum medical improvement in this office. The patient will continue with home care instructions if symptoms persist.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 01/14/2025 11:10 AM

Addenda

The patient is being discharged today after a course of care with residual deficits in function from the injuries sustained from their Motor Vehicle Accident.
According to the American Medical Association (AMA) Guides to the Evaluation of Permanent Impairment 5th edition this patient may have a Permanent whole-body impairment of up to 25%.
In order to prevent further damage this patient will have to receive regular treatment at the rate of 2 times per month for life.
Approximate $104,940 in conservative lifetime treatment costs.

Lifetime Treatment Costs = [Avg. Life expectancy - Current Age] X 12 X $150= Lifetime treatment.

The average Male Life Expectancy is 75.3 years.
The average Female Life Expectancy is 81.1 years.

Electronic Signature: Heinis, Ricardo DC

Added on Jan 14, 2025